

[ENTER COMPANY NAME]

QUALITY MANAGEMENT SYSTEM

STANDARD OPERATING PROCEDURE

Internal Quality Auditing Program

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Applicable Regulatory Standards	ISO 13485:2016 Clause 8.2.4 FDA 21 CFR Part 820.22 ISO 19011:2018 (Auditing Guidelines)

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1.0 Purpose

This Standard Operating Procedure (SOP) defines the mandatory operational framework for scheduling, planning, executing, documenting, and closing internal quality audits within the organization. The primary objective is to evaluate the continuous suitability, adequacy, and operational compliance of the Quality Management System (QMS) against established standards, isolate process efficiencies or vulnerabilities before external inspections, and provide management with actionable insights for system continuous improvement.

2.0 Scope

This procedure is universally applicable to all operational departments, manufacturing cells, cleanroom zones, facilities, and product-specific lines that support or define the QMS. The auditing scope covers all clauses of ISO 13485, FDA 21 CFR Part 820, and applicable international regional medical device directives.

3.0 Responsibility and Authority

3.1 Quality Assurance Manager / Audit Program Manager

Holds organizational authority to compile the Annual Internal Audit Schedule, select qualified audit teams, track findings, and report systemic noncompliance trends directly during formal Management Review sessions.

3.2 Lead Internal Auditor

Responsible for formulating area-specific audit plans, executing objective process evaluations, evaluating physical evidence, categorizing findings, and generating formal Internal Audit Reports.

3.3 Auditee / Departmental Process Owners

Responsible for providing the audit team with unrestricted access to physical cleanroom spaces, quality records, electronic tracking systems, and personnel. Process owners are accountable for executing prompt corrective actions for any identified noncompliance items.

4.0 Procedure

4.1 Formulating the Risk-Based Annual Audit Schedule

The Audit Program Manager must establish a prospective Annual Quality Audit Schedule. While every QMS subsystem must be audited at least once every twelve (12) months, the scheduling frequency for individual areas must be risk-prioritized based on the following criteria:

- The complexity of the underlying technology or production process (e.g., sterilization and cleaning lines are prioritized).
- Historical process performance parameters, scrap trends, and material nonconformity frequencies.

- The volume and severity of post-market clinical complaints or vigilance alerts targeting the area.
- The volume and history of previous audit findings or external inspectorate deficiencies.

4.2 Principles of Auditor Independence

To preserve complete objectivity and legal validity, internal quality audits must be conducted by certified internal auditors who possess complete independence from the specific department or process undergoing evaluation. Auditors are strictly prohibited from evaluating their own operational work or the immediate workflows under their direct supervisory hierarchy.

REGULATORY SURVEILLANCE COMPLIANCE: In accordance with FDA 21 CFR 820.22, detailed internal audit reports are exempt from routine FDA investigator review. However, the organization must provide objective evidence (e.g., the Master Audit Schedule, closed finding checklists, and CAPA logs) proving that the internal quality audit program is operating continuously, systemically, and effectively.

4.3 Phased Execution Workflow of an Internal Quality Audit

Every individual audit assignment must advance through an unyielding, structured operational progression:

Phase 1: Pre-Audit Planning and Checklist Formulation

The Lead Auditor reviews applicable SOPs, regulatory criteria, and historical files. The auditor Populates a formal Internal Audit Checklist detailing the precise sampling strategies and process controls to challenge.

Phase 2: Opening Meeting and Process Evaluation

Convene a meeting with the department owners to clarify the scope. The audit team executes live field walks, evaluates real-time cleanroom parameter logs, reviews electronic records via a random sampling approach, and interviews line operators.

Phase 3: Grading of Findings and Closing Session

The audit team compiles the evidence, isolates any systemic noncompliance, and categorizes findings based on severity rules. Conclude with a closing session to align on the findings with department process owners.

4.4 Classification Metrics for Quality Audit Findings

Identified system noncompliance must be objectively categorized using standard severity rules to drive appropriate remediation urgency:

Finding Classification	Technical and Regulatory Impact Definition	Required CAPA Trigger Deadline
Major Noncompliance	A systemic breakdown or total absence of a mandated QMS process or regulatory clause requirement, or an escape that directly compromises product safety or clinical efficacy.	Mandatory CAPA file opened within 5 business days. Full root-cause analysis required.
Minor Noncompliance	An isolated, spot clerical lapse, minor technical tracking variance, or non-systemic drift that does not compromise overall device function or patient safety.	Corrective action plan required within 15 business days. CAPA evaluated per case.
Opportunity for Improvement (OFI)	A technically compliant process layout that exhibits vulnerability to future degradation or represents an opportunity to align with state-of-the-art efficiency models.	No mandatory CAPA required. Process owner reviews for potential optimization.

4.5 Reporting, CAPA Linkage, and Audit Program Closure

The Lead Auditor must issue the final formal Internal Audit Report within ten (10) business days of the closing meeting. The auditee must populate the remediation response blocks, detailing explicit root-cause investigations, containment profiles, and corrective action completion timelines. The auditor must formally verify the implemented fixes—by auditing physical evidence or performing dynamic field checks—before changing the specific internal audit record status to "Closed".

5.0 Documentation Archiving and Records Retention

All finalized annual schedules, area checklists, raw field notes, signed audit reports, and verification documents are critical quality records. They must be archived securely for a minimum duration of ten (10) years from the date of file closure, and made available to international notified bodies and registrars during system surveillance checks.

6.0 Associated Reference Materials

- **SOP-QA-001:** Document and Data Control System
- **SOP-QA-006:** Corrective and Preventive Action (CAPA) System
- **Form-QA-016A:** Annual Internal Quality Audit Schedule Log
- **Form-QA-016B:** Internal Audit Report and Finding Response Template

7.0 Revision History Log

Revision Level	Effective Date	Author / Originator	Detailed Description of Modifications
00	[Date]	[Name], Quality Systems	Initial baseline release of formalized Internal Quality Auditing Program Standard Operating Procedure.